

Every contact counts: a call for inclusion of meaningful relational maternity care in low-income and middle-income countries

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Within maternity services, much emphasis has been placed on continuity of care and continuity of carer as key conduits for trusting relationships and positive experiences, which are essential for improved outcomes. However, continuity models remain aspirational in most settings, with national scale-up being a rarity. At the crux of the philosophies that support these approaches is relational care, something that should be achievable regardless of context. Relational care supports quality of care by emphasising mutuality of trust, effective two-way communications, and shared decision making. However, the development of a therapeutic relationship between health-care provider and health-care recipient, a key element of relational care, has been identified as a particular challenge in low-income and middle-income countries (LMICs). Using an iterative, narrative approach, we reviewed the literature on relational care within maternity settings, with a focus on LMICs. We explored experiences of relational care in LMICs, as well as obstacles and facilitators to achieving it and the effects of relational care across the maternity continuum.

Introduction

Relational care (or relationship-based care) is defined as a type of care “where the patient is cared about, as well as cared for, and where the professional patient interaction is reciprocal – they give as well as receive, where they share in the suffering of those they care for”.¹ This approach builds on the more traditional person-centred care by moving beyond the individual to acknowledge social and relational aspects.² Thus, where people have strong, trusting relationships with health workers, this approach facilitates person-centred care and shared decision making. In a relationship-centred approach, the input, individuality, experiences, and perspectives of the health-care provider are also recognised as fundamental contributors. The ethos of relational care rejects the opposing tenets of transactional care, which prioritises efficient completion of tasks, measuring quality based on adherence to protocols, and working within set budgets.¹ Although important for safe delivery of services, focusing solely on the technical aspects of health care negates the human connections and social needs of health-care recipients and providers. Crucial to relational care is reciprocity,^{2,3} whereby all involved parties contribute and gain, non-materially, from the relationship. For example, health-care recipients can receive supportive communications and feel respected,⁴ whereas staff can benefit from feeling valued or having an empathic understanding.⁴

There has been a growing acknowledgment of the importance of relational care in maternity services during the past five decades, as evidenced by its inclusion in international policies, for example in the latest WHO maternal and newborn recommendations.^{5,6} However, within the academic literature, relational care is often conflated with continuity of carer, whereby the same midwife or group of midwives builds a relationship with a woman and her family from pregnancy until the

postpartum period.⁷ A scoping review of 175 reports found that only one country, New Zealand, has been able to scale up continuity of midwifery care at a national level.⁸ We would argue that, although repeat contacts can provide an optimum scenario, longer relationships are not always functional and short contacts not necessarily superficial (figure). Importantly, although continuity of carer is, by nature, relational, relational care is possible without continuity of carer.

Relational care could, and should, occur during every interaction between a health worker and health-care recipient. Such relational care is particularly important in low-income and middle-income countries (LMICs), where traditional continuity of carer models might not be feasible or sustainable due to scarce human resources and poor infrastructure.⁹ Following the maternity continuum, we explored the literature on relational care during three timepoints: a woman's first contact with maternity services, during pregnancy, and throughout birth. Our focus was on LMICs; however, we acknowledge that some of the issues are also relevant to high-income countries.

First contact with maternity services

One of the most crucial interactions in maternity care is the first antenatal contact. For some women, this contact is their first contact with health services, and experiences have enduring influence on satisfaction with care.¹⁰ The first visit is essential for early identification of modifiable risk factors and pre-existing complications,¹¹ and an opportunity for health promotion.

However, in many instances, what has been described as a tick-box culture prevails,¹² and health workers need to address each item on an antenatal card or schedule within a restricted allotted time slot. Thus, care tends to be transactional; the health worker prioritises screening and identifying any issues, and the pregnant woman is treated

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Concepts	Definition	Relational care	Person-centred care	Respectful care	Continuity of carer	Bidirectional interplay
Relational care	Care focused on reciprocal health provider–recipient interactions	✓	✓	✓	✗	✓
Person-centred care	Care focused on individual needs and preferences	✗	✓	✓	✗	✗
Respectful care	Care focused on treating everyone with dignity, compassion, and empathy	✗	✓	✓	✗	✗
Continuity of carer	Care provided continuously by the same health-care provider or team	✓	✓	✓	✓	✓

Figure: Relationships between the key concepts of maternity care

according to the findings. This approach can leave a woman feeling undervalued, affecting their future engagement with maternity services during the index and subsequent pregnancies.¹³ Furthermore, negative maternity experiences are likely to be shared within the community, thus influencing the health behaviours of others¹⁴ and deterring some from antenatal engagement.¹⁵ Transactional care can also have a negative effect on health workers. They place great importance on the relational aspects of their jobs, which can be a catalyst to job satisfaction and loyalty to the organisation.¹⁶

Exploring the aspects of care that can make a difference to pregnant woman have revealed consistent aspects, including a friendly welcome, courteous language, and meaningful information exchanges. For example, in Mdoe and colleagues' large qualitative study,¹⁷ conducted in Malawi and Tanzania, women and family members stated that they felt well received, and health-care providers recognised the importance of greeting women without judgement, introducing themselves by name on first contact, using the woman's preferred name, and maintaining a calm tone.¹⁷ Use of non-verbal behaviours, such as smiling and eye contact, were also highlighted as contributing to supportive care. These positive behaviours can be the catalyst to relational care, moving beyond respectful maternal and newborn care,¹⁸ which focuses on strengthening individuals' experiences, rights, and autonomy to develop an authentic partnership that also emphasises mutual trust, shared decision making, and a strengthened relationship.¹⁹

However, conversations initiated by health workers often do not consider health literacy, meaning that people with low health literacy are often unable to engage in the discourse.²⁰ Furthermore, untailored, so-called information dumping often takes place at the first antenatal visit, which could overwhelm some women, preventing them from

having a meaningful interaction.²¹ The limited relational agency identified in Kenya, for example, prevents effective communication and reduces a person's autonomy.²²

Care during pregnancy

Relational care throughout pregnancy is absent in many LMICs, even when continuity of carer is practised. We have observed that many women attend the same facility and are assessed by the same health workers, but they receive care that does not go beyond clinical tasks and treatment. This observation is supported by Fentie and colleagues²³ study conducted in Ethiopia, which found that the health workers' focus on tasks is often at the expense of the broader relational needs of the health-care recipient. A grounded theory study of 98 participants in Zambia and Tanzania also supports our observation, highlighting that even when a high standard of clinical care was provided, relational care was scarce.⁴ Importantly, this study found that relational care was provided discriminately, with health workers displaying affinity bias²⁴ in which they would only provide relational care to individuals they could identify with (eg, same tribe, education, or social status).⁴ These studies show that relational care is possible.

Of concern is the influence of women's experiences of relational care on antenatal assessments and subsequent care. For example, qualitative data derived from interviews with women in Ethiopia found that they were unwilling to reveal psychosocial issues with health workers who did not communicate empathically.²⁵ An additional barrier to provision of relational care is that it can be undervalued by health-care recipients and workers.⁴ In our previous work,⁴ conducted in Zambia and Tanzania, health workers prioritised clinical treatment and women were prepared to forgo relational care if they believed this treatment would benefit them and improve their pregnancy outcome.

Although women would have liked optimum clinical care and meaningful relational care, they did not expect this care to be an option and saw it as an “added extra”.⁴ These findings resonate with a mixed-methods study in Ethiopia, which found that women’s expectations of care were low, as was their agency to demand changes.²⁵

Care during birth

A woman will remember their labour and birth experience, whether positive or negative, for many years. Pivotal to this experience is their relationship with the attending health worker.²⁶ A competent health worker will follow protocols and ensure that correct treatment is provided. However, this is not enough; people deserve care that supports emotional and physical safety.²⁷ When health workers and people can laugh²⁸ and cry²⁹ together, it is a good indication of a reciprocal relationship. Such relational exchanges can increase the trust between midwives and pregnant women and, in addition, can build confidence in women’s ability to give birth,³⁰ resulting in positive outcomes and experiences.³¹

However, in some settings, numerous barriers exist to preventing relational care during birth. The absence of a relational environment is one factor, whereby the organisational culture asserts that treatment, procedures, and processes are most important, and tasks dominate the interactions. For example, a study in South Africa³² described “relationally hostile and emotionally distressing birthing environments”, which excluded the elements that are important to women in labour such as a positive relationship with health workers, decision-making powers, labour support, and the presence of a birth companion.³² In many LMICs, a hierarchical relationship prevails, whereby the health worker is in control, makes all the decisions, and communicates on a need-to-know basis.³³ This hierarchical relationship marginalises a person, makes them feel insignificant, and forces them to submit to the requests of the health worker.³⁴ This was powerfully highlighted in a qualitative study with 41 women in their postnatal phase in Ethiopia,³⁴ wherein participants remained silent, even when insulted and treated badly. Sadly, women expected care that was “technical and unidimensional”,³⁴ and these expectations were met; a dependency on health workers to provide a service that resulted in a live mother and baby resulted in acceptance of transactional care. In addition to the relational environment, the physical environment can also be a deterrent to health worker–recipient relationships. Women birthing in communal areas, often without screens or curtains, and people passing by hinders focused individualised interactions.²⁶ One report from Tanzania³⁵ referred to the labour ward as “a hall”, with as many as 20 strangers being able to observe a woman in labour as they passed by. This issue is likely to be further compounded by the number of student and junior medical observers that often are present during a birth. Although learning opportunities are important, we are aware, from our own observations, of as many as 12 students observing

a birth in LMICs, none of whom introduced themselves to the woman or tried to develop a relationship.

Most available evidence on transactional care in LMICs stems from public facilities,^{4,23,34} which are often associated with staff shortages and poor resources. However, attending a private facility does not necessarily result in receipt of relational care. One interview study³⁶ of 40 women attending a private obstetric practice in Thailand found that they believed their payments would result in mutual trust between them and the obstetrician. However, obstetricians seldom acknowledged the relationship as reciprocal and did not provide the relational care expected by women, leaving women disappointed and likely to complain. This finding reinforces the notion that relational care is independent of the resources available. The most powerful influence is the culture of the organisation, with entrenched beliefs and practices maintained through sociocultural influences and power differentials.

Recommendations and call for action

The systems-level reforms required to implement and scale up continuity of care and a lack of evidence-informed models adapted for resource-constrained settings reduce access to the benefits of relational care for people in LMICs. A promising strategy with potential short-term and medium-term gains would be to optimise relational interactions in every care episode. Although the depiction as relational care has been disputed, there are numerous reports of impactful encounters that have connected a midwife and pregnant woman during a single episode of care.³⁰ Such interactions linger in women’s and midwives’ memories for a long time and can influence future health-seeking behaviour, including in others.³⁷ We would, therefore, call for the status of relational care to be elevated.

First, a shift in the education and preparation of health workers in LMICs will be required to ensure that the meaning and principles of relational care are understood. Although midwifery curricula in LMIC settings have been adapted to focus on individual experience and wellbeing for respectful maternal and newborn care, emphasis on the quality and dynamics of relationships between health-care providers and recipients is still not consistently prioritised.^{38,39} At the individual health-worker level, creation of conditions supporting effective relational care requires understanding of behavioural drivers. Sensitive and compassionate verbal and non-verbal communication and using opportunities in any care contact to create relationships are key. Skills required include active listening, showing empathy, and providing understandable information to enable decision making.⁴⁰ Strengthening both pre-service and in-service education to ensure health workers develop and continuously improve these abilities, including cultivating self-awareness and understanding of the needs of women and families in their care, will be necessary. These behaviours will also support improved maternal health literacy, which, in turn, can reduce health-care provider–recipient power imbalances.²⁰ Development

Search strategy and selection criteria

We conducted a narrative review of the literature. We initially searched PubMed and Cumulative Index to Nursing and Allied Health Literature from database inception to April 30, 2025, using the search terms “relational care” OR “transactional care” AND “maternity” AND “LMIC”, with synonyms and appropriate truncations, in titles and abstracts. We then used a non-linear process to enable a more dynamic, iterative approach using reference lists of retrieved papers, including relevant grey literature. Papers were limited to English or Swahili, with no restrictions on research methodology. Papers were included on the basis of topic relevance, decided by all authors.

opportunities not only improve the quality and effectiveness of communication but also have important wider benefits for health workers in LMICs. Studies in Ethiopia⁴¹ and Tanzania⁴² have shown increased job satisfaction, motivation, and retention of health workers in maternity care.

Second, improving individual health workers’ practice in isolation is unlikely to create an environment in which relational maternity care can flourish. The physical environment of care and organisational culture are also crucial.⁴³ Aikins and colleagues, in their multicentre study in Ghana,⁴⁴ showed the importance of an enabling emotional and physical environment for health workers’ job satisfaction and subsequent care delivery. Care environments that do not have privacy, compromise confidentiality, or are noisy or uncomfortable are most likely to affect provider–recipient contacts and disrupt relational and respectful care. Cost-effective strategies are needed to overcome design, layout, and physical-space barriers in many facilities in low-resource settings. Support for women and families who are not proficient in the local language or have specific communication needs (eg, hearing disability or neurodisability) will also need to be considered.

Finally, power imbalances, poor provider–manager communication, and absent or punitive supervision have been identified as pervasive influences on experiences and quality of care in LMICs.⁴⁵ Transition from hierarchical organisational culture to positive relationships and supportive leadership is important to build and sustain relational-care capacity. Evidence from health-care and LMIC settings is scarce. However, a 2023 ethnographic study in UK social care showed the particular importance of staff and staff–management relationships, with collaborative communication, feeling valued, and respect for work–life balance perceived as enabling factors.⁴³ A review of studies in LMICs emphasised that the process of shifting organisational culture could not be addressed solely by training, and should include group action to identify and address workplace issues involving all staff cadres and grades.⁴⁶ Ideally, the perspectives of key stakeholders and communities should also be included. Among participatory approaches, appreciative inquiry, a strengths-based method that supports organisational change,⁴⁷ has potential advantages. In this model, the focus is on identifying and building on existing good practice, encouraging

collaboration and motivation, by contrast with traditional problem-focused methods that can accentuate focus on weaknesses, generating criticism and division.

Conclusions

Regardless of the country setting, resources, midwifery model, or ability to deliver continuity of care or carer, relational care should be possible. It is so much more than a long-term relationship between a woman and health worker; it is a positive encounter at each contact, whereby all parties gain from the experience. Overcoming some of the barriers to relational care is likely to be low cost or cost neutral, and resources should not be used as an argument for transactional care. Instead, organisations should foster a reciprocal approach, whereby health-care recipients and workers gain from the interactive experience. Adopting an appreciative inquiry approach by valuing positive behaviours, facilitating training, and ensuring a relational environment should be considered. Such an approach not only benefits health-care recipients but also health workers and the organisation as a whole.

Contributors

TL and TAM conceptualised the idea for the manuscript and discussed with IC and LN. TL and TAM reviewed the literature. TL wrote the first draft, with input from TAM. IC and LN provided feedback on the manuscript, offering important intellectual input and contextual information. TL revised the first draft with input from all authors. All authors reviewed and approved the manuscript and had final responsibility for the decision to submit for publication.

Declaration of interests

We declare no competing interests.

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